

2025

GENERATION P

2025

Annual Report

01 / 2025 AT A GLANCE

Who we are

More than half of young adults aged 18 to 34 live with at least one chronic or rare condition, yet this group remains largely overlooked in healthcare, research, and policy.

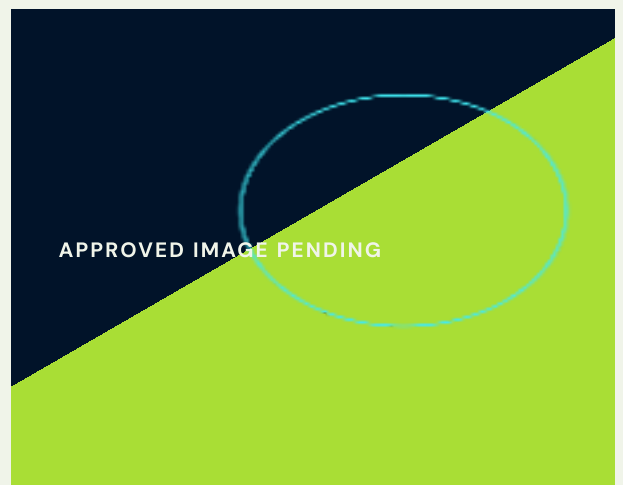


IMAGE PLATE / CAPTION AND CREDIT OCCUPY THIS LINE WHEN SUPPLIED

Built by the people living inside the gap.

Generation Patient was built to close that gap by the people living inside it. Our founder started this organization as a young adult with a chronic illness, and young adult patients still lead it at every level, from our peer facilitators to our policy scholars to our board. We know these systems firsthand, and that is exactly why we can change them.

EVIDENCE RAIL

Patient-led at every level

Peer facilitators

Policy scholars

Board

A / COMMUNITY

Direct support

Community-led peer support groups
Fellowship and leadership
programming

Direct support informs
systems change, so lived
experience drives lasting
reform.

B / REFORM

Systems change

Four Health Policy Priorities
Multi-stakeholder roundtables
Peer-reviewed publications

Every supplied result. *No dashboard.*

01	DIRECT SUPPORT	77 Peer support groups	Convened 77 peer support groups serving more than 115 young adult patients, with 17 trained community facilitators, and expanded internationally with new groups in South Asia and East Africa co-hosted with local patient advocacy partners.
02	DIRECT SUPPORT	100% Less alone	100% of survey respondents said our peer groups helped them feel less alone, and 100% were very or mostly satisfied with group structure, facilitation, topics, and accessibility. We continually seek feedback to make the groups more accessible and supportive for our young adult patient community.
03	DIRECT SUPPORT	80+ Fellow projects	The Crohn's & Colitis Young Adults Network (CCYAN), our only disease-specific program, marked its seventh year by supporting 8 fellows across the U.S., India, Ethiopia, and Bangladesh, who produced 80+ articles, videos, and advocacy projects. Every fellow rated the fellowship 5/5, and the alumni network now spans 50 fellows worldwide.
04	SYSTEMS CHANGE	Patent reform	Helped advance the reintroduction of the bipartisan Eliminating Thickets to Increase Competition (ETHIC) Act to curb patent thickets and lower drug prices; our Executive Director's statement was featured in the official Senate press announcement.
05	SYSTEMS CHANGE	100 FDA enforcement letters	Our years-long advocacy on deceptive pharmaceutical advertising contributed to concrete enforcement by the U.S. Food and Drug Administration (FDA), which issued 60 warning letters and 40 untitled letters to companies engaged in misleading drug marketing on social media.
06	SYSTEMS CHANGE	AI companion oversight	Delivered formal remarks to the FDA's Digital Health Advisory Committee urging that AI companion tools claiming to address mental health be regulated as medical devices.

07	SYSTEMS CHANGE	Clinical trial representation	Met regularly with FDA leadership and engaged in the Prescription Drug User Fee Act (PDUFA), Medical Device User Fee Amendments (MDUFA), and Generic Drug User Fee Amendments (GDUFA) processes to push for age-disaggregated clinical trial data, better representation of young adults in research, and stronger regulatory rigor.
08	SYSTEMS CHANGE	7 Roundtables	Convened seven invite-only roundtables bringing young adult patients together with nationally leading clinicians, researchers, and policymakers, each of which will result in a peer-reviewed publication first-authored by a young adult patient. The first three were published in 2025.
09	SYSTEMS CHANGE	Peer-support evidence	Published a mixed-methods peer-reviewed study on our peer support work, co-authored and first-authored by young adult patients.
10	SYSTEMS CHANGE	Research guidance	Completed a multi-year roundtable funded by a Patient-Centered Outcomes Research Institute (PCORI) Eugene Washington Engagement Award, producing new national guidance, a researcher checklist, and a toolkit for peer support research for young adults with chronic and rare conditions.
11	SYSTEMS CHANGE	Global consensus	Launched a first-of-its-kind global consensus initiative at the Commonwealth Fund on the sidelines of the United Nations General Assembly in New York City. The published proceedings will lead to the first shared definition of young adults with chronic conditions and non-communicable diseases (NCDs).

Figures and causal claims remain subject to the checks in editorial-review.md.

03 / THANK YOU

Thank you for
believing that
young adult
patients should
lead the change
we need.

GENERATION PATIENT DOES NOT ACCEPT FUNDING FROM THE PRIVATE HEALTHCARE INDUSTRY.

In 2026, we will scale that momentum.

-
- 01 Expand peer support capacity to meet growing demand.
 - 02 Deepen policy impact across four priorities.
 - 03 Advance the authentic patient voice through transparency.
 - 04 Publish and facilitate adoption of the global proceedings.
-

Because we refuse industry funding, every dollar we raise comes from people and foundations who share our mission.

Consider making a donation ↗

05 / OUR WORK IN DEPTH

Peer Support

Peer support is one of the most powerful tools in healthcare, and one that has not yet been taken as seriously as it deserves.

77 GROUPS / 115+ PATIENTS / COMMUNITY-LED

Community is evidence.

Young adults with chronic and/or rare conditions experience disproportionately high rates of isolation, anxiety, depression, and disruption to education, employment, and enjoyment of everyday life. Despite being at a pivotal stage of life and development, the mental well-being of young adults with chronic medical conditions is frequently overshadowed by our physical health needs. We believe that peer support is one crucial and often overlooked resource that can help reduce isolation and increase feelings of validation, acceptance, and connection in our community.

Since March 2020, we have facilitated more than 650 virtual peer support meetings, all led by facilitators who are themselves young adults living with chronic conditions. Most groups are cross-disease, reflecting the shared experiences of school, career, and relationships that cut across diagnoses.

2025 GROUPS

23 general

18 higher education

20 IBD

15 international

Listed categories total 76; the supplied headline total is 77.

IN OUR COMMUNITY'S WORDS

“There have been weeks when my only laughter came from the peer support meetings.”

“I show up with my camera off on high-symptom days and still feel completely included. That flexibility keeps me coming back.”

“I didn't realize how much I was carrying until I heard other people describe exactly what I've been feeling.”

Four priorities. *Four dossiers.*

01

Patent reform

Advancing competition and access to more affordable medicines.

02

Deceptive drug ads

Pushing for oversight of misleading pharmaceutical marketing online.

03

Clinical trials

Making young adults visible in research, data, and study design.

04

AI companion tools

Calling for evidence, regulation, and post-market oversight.

01

Patent reform

We helped support the reintroduction of the bipartisan Eliminating Thickets to Increase Competition (ETHIC) Act, led by Senators Welch, Hawley, and Klobuchar, which would limit pharmaceutical companies to asserting a single patent from a thicket of overlapping patents to ensure more timely access to generics and biosimilars. We also advocated against the Patent Eligibility Restoration Act (PERA) and the Promoting and Respecting Economically Vital American Innovation Leadership Act (PREVAIL) Act, which would both create barriers to affordable medicines and prioritize profits for the pharmaceutical industry, and met with dozens of congressional offices. We were also invited to speak at the DOJ and FTC Listening Session on anticompetitive conduct in the pharmaceutical industry, convened under a presidential executive order on lowering drug prices, where our testimony informed the agencies' joint report on pharmaceutical antitrust submitted to the White House.

“Young adult patients are living with the consequences of patent abuse every day. These policies decide whether people can afford the medications they need to stay alive.”

ALEXANDER NAUM, JD
/ POLICY MANAGER

Deceptive drug advertising

Nearly 90% of young people use social media to find health information, where they face misleading pharmaceutical marketing designed to exploit their vulnerabilities. We have been a leading voice behind the bipartisan Protecting Patients from Deceptive Drug Ads Act, and in 2025 our advocacy contributed to the FDA issuing 60 warning letters and 40 untitled letters to companies engaged in deceptive marketing, including undisclosed influencer promotions.

EVIDENCE RAIL

Approved links, testimony, legislation, and agency references occupy this fixed component slot.

Clinical trials

Trials often lump everyone aged 18 to 64 into one category, obscuring the distinct needs of young adults. We pushed for age-disaggregated trial data, better representation of young adults in study design, improved adverse-event reporting, and rigorous approval standards, meeting routinely with FDA leadership and engaging in the Prescription Drug User Fee Act (PDUFA), Medical Device User Fee Amendments (MDUFA), and Generic Drug User Fee Amendments (GDUFA) processes. These user fee agreements determine how industry fees fund the FDA's review work and are renegotiated with stakeholder input, making them a critical lever for patient-centered reform.

EVIDENCE RAIL

Approved links, testimony, legislation, and agency references occupy this fixed component slot.

AI companion tools

AI tools marketed as emotional or mental health support increasingly target young people, including those with chronic illness. We delivered formal remarks to the FDA's Digital Health Advisory Committee urging that these tools be regulated as medical devices when they claim to address mental health and submitted comments outlining the current regulatory gaps. We are working to require robust clinical evidence before marketing, clear regulatory pathways, and proactive post-market surveillance. This is our newest policy priority and one we will expand significantly in 2026.

EVIDENCE RAIL

Approved links, testimony, legislation, and agency references occupy this fixed component slot.

COMMUNITY SPOTLIGHT

Peyton Miles

In March 2022, during her senior year of high school, a viral illness left Peyton Miles with a severe, unremitting headache and, soon, a life lived from bed while her friends attended prom and graduated.

APPROVED PEYTON MILES PORTRAIT PENDING

“Generation Patient didn't just give me an idea of how I could use my experience to create change; it gave me a purpose and a community, which was so much more than I could've asked for.”

COMMUNITY SPOTLIGHT / CAPTION AND CREDIT

Roundtables connect lived experience to decisions.



Each session will result in a peer-reviewed publication first-authored by a young adult patient. The first three papers were published in 2025.

Media coverage + donors

A / SHARED RULED FAMILY

01 Welch, Hawley,
 Klobuchar
 Introduce
 Bipartisan
 Legislation to
 Streamline Drug
 Patent Litigation

OFFICE OF SENATOR PETER
WELCH ↗

02 First Listening
 Session on
 Lowering Drug
 Prices Through
 Competition:
 Official
 Transcript

FEDERAL TRADE
COMMISSION ↗

Arnold Ventures	Commonwealth Fund
Connecting to Cure Crohn's	Disability Frontlines Fund

B / INDEPENDENT TREATMENTS

DRUG PRICING AND
PATENT REFORM

Welch,
Hawley,
Klobuchar
Introduce
Bipartisan
Legislation to
Streamline
Drug Patent
Litigation

OFFICE OF SENATOR
PETER WELCH ↗

OFFICIAL TRANSCRIPT

First
Listening
Session on
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